

Policy on Ambulance Services

«Hospital Name»

Document control

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Prepared by	Ambulance/Transport In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

«Hospital Name» shall have access to ambulance services commensurate with its scope of services. Ambulance services shall support safe patient transport with appropriate care, adequate access and space, appropriate equipment and trained personnel, reliable communication, and safe transfer of clinical information.

1. Purpose

The purpose of this policy is to establish requirements for ambulance access, space, statutory fitness, equipment, personnel, daily and post-trip checks, communication and support for patients while in transit.

2. Scope

This policy applies to ambulance services used by «Hospital Name», whether provided in-house or through an outsourced service.

3. Policy standards

3.1 Ambulance access and level

The organisation may provide an in-house or outsourced ambulance service for safe patient transport with appropriate care.

The organisation shall decide the appropriate level of ambulance based on the National Ambulance Code AIS-125.

3.2 Ambulance space and access

The organisation shall demarcate a proper space for ambulances, keeping easy accessibility for receiving patients and enabling ambulances to exit quickly.

Adequate and prominent signage shall guide ambulance drivers to the ambulance entry and route to the emergency department.

It is preferable that the organisation has an ambulance parking bay.

3.3 Fitness and equipment

The vehicle used as an ambulance shall adhere to applicable statutory requirements. Examples include registration as an ambulance under the Motor Vehicle Act, a valid fitness certificate, pollution control certificate and insurance. These are examples and not an exhaustive list of statutory requirements.

An ambulance shall have at least basic life support equipment for both adult and paediatric patients.

Based on the organisation's scope, additional equipment, for example monitoring and resuscitative equipment, shall be available in the ambulance.

Reference: National Ambulance Code AIS-125.

3.4 Personnel

The ambulance shall be operated by a driver with a valid driving licence.

Personnel in the ambulance shall have training in basic life support and basic cardiopulmonary resuscitation.

A technician, nurse and/or doctor may be present depending on the situation and scope of the ambulance.

3.5 Daily and post-trip checks

The daily check shall indicate the functioning status of ambulance systems like lights, siren, beacon lights, fuel and tyres.

Medical equipment shall be checked using a documented checklist.

Emergency medications shall be available in the ambulance during patient transport.

A daily check shall ensure availability and expiry dates of emergency medications and shall be documented.

After every trip, medications used shall be topped up if used and the same verified and documented.

Medications shall be stored in a safe environment as per organisational policy.

If the organisation follows a system of sealing the emergency medication kit, the check shall be carried out after each use of the kit.

3.6 Communication

The ambulance shall be connected with the organisation/control room by wireless/mobile phones.

The communication system shall encompass the whole process of patient transport.

There shall be written guidance on how a call for patient transport is received, who is expected to respond and who organises the transport.

Communication shall ensure that the ambulance leaves the hospital within a predefined timeframe based upon the patient's needs, as documented in the Ambulance Operations SOP maintained by the Ambulance/Transport In-Charge.

3.7 Care while in transit

From first communication with the patient or attendant, a file is created to record appropriate information.

Attempts are made to gather important clinical information such as age, weight, provisional diagnosis and ongoing treatment at the referring organisation.

This information is used by ambulance personnel of the receiving organisation to be better prepared to assess, initiate emergency care/interventions during transit and transport the patient safely.

During transit, when required, there is an exchange of information between ambulance personnel and the medical professional at the receiving organisation.

When the patient is shifted by an external agency, wherever possible, the doctor of the receiving organisation attempts to communicate with the ambulance personnel of the external agency to ascertain the clinical situation and make appropriate suggestions.

The medical professional in the ambulance is responsible for decision-making regarding care/interventions during transit.

4. Non-negotiable rules

- Do not dispatch or continue an ambulance transfer when the vehicle fails its readiness check, required equipment is non-functional, the driver is not licensed for the vehicle class, or the organisation/control-room communication link is down.
- Do not operate an ambulance without at least basic life support equipment for both adult and paediatric patients.
- Do not operate an ambulance without a driver with a valid driving licence.

- Do not allow ambulance personnel to provide service without basic life support and basic cardiopulmonary resuscitation training.
- Do not omit the daily check of vehicle systems, medical equipment and emergency medication availability and expiry, or the required documentation.
- Do not omit the post-trip top-up of medications used, verification and documentation.
- Where a sealed emergency medication kit is used, do not omit the check after each use.
- Do not operate the ambulance without the required organisation/control-room communication link, whole-process communication coverage and written call-receipt/response guidance.
- Do not dispatch an ambulance outside the predefined timeframe based on the patient's needs.

5. What we do

5.1 Access and ambulance level

Provide in-house or outsourced ambulance service as decided by the organisation.

Select the appropriate ambulance level based on National Ambulance Code AIS-125.

5.2 Space and access

Demarcate ambulance space for easy patient receipt and quick exit.

Maintain prominent signage to the ambulance entry and emergency department route.

An ambulance parking bay may be provided.

5.3 Vehicle fitness and equipment

Maintain statutory compliance applicable to the ambulance.

Provide at least basic life support equipment for adult and paediatric patients.

Provide additional monitoring and resuscitative equipment based on scope.

5.4 Personnel

Use a driver with a valid driving licence.

Ensure ambulance personnel have basic life support and basic cardiopulmonary resuscitation training.

A technician, nurse and/or doctor may be included depending on situation and scope.

5.5 Checks and medication

Perform and document the daily vehicle, medical equipment and medication checks.

After every trip, top up medications used, verify and document this.

Where a sealed kit system is used, check the kit after each use.

Store medications safely according to organisational policy.

5.6 Communication

Maintain connection with the organisation/control room by wireless or mobile phones.

Ensure communication covers the whole transport process.

Follow written call-receipt and response guidance.

Ensure ambulance departure within the predefined timeframe based on patient needs.

5.7 In-transit care

Create a file from first communication and gather appropriate clinical information.

Use available information to prepare for assessment, emergency care/interventions and safe transport.

Exchange information with the receiving medical professional when required.

Where an external agency transports the patient, wherever possible, the receiving doctor attempts communication with its ambulance personnel.

The medical professional in the ambulance makes decisions regarding care/interventions during transit.

6. Stop-work authority

Do not dispatch or continue an ambulance transfer when the vehicle fails its readiness check, required equipment is non-functional, the driver is not licensed for the vehicle class, or the organisation/control-room communication link is down. Stop-work applies to the transfer dispatch, not to on-scene life-saving measures. The person who stops tells the Ambulance / Transport In-Charge and the Medical Superintendent the same shift. Refusing an unsafe ambulance dispatch is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Ambulance/Transport In-Charge	Ambulance/Transport In-Charge oversees ambulance readiness, dispatch, communication and transport processes.
Management	Management determines the appropriate ambulance level and whether services are provided in-house or outsourced.
Ambulance personnel	Ambulance personnel maintain required checks, equipment, medications and

	communication.
Medical professional in ambulance	The medical professional in the ambulance is responsible for decision-making regarding care/interventions during transit.

8. Quality monitoring

Monitoring area	What is monitored
Daily readiness checks	Daily ambulance readiness checks and documented findings
Post-trip checks	Post-trip medication top-up, verification and documentation
Equipment and medication	Equipment functionality and medication availability/expiry
Communication	Communication-system availability and predefined response timeframe
Access and space	Ambulance access, space and signage
Statutory compliance	Compliance with applicable statutory requirements

9. Training and staff acknowledgement

Ambulance personnel shall have basic life support and basic cardiopulmonary resuscitation training. The ambulance driver shall have a valid driving licence.

I have read the Policy on Ambulance Services of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to ambulance/transport personnel and personnel responsible for emergency patient transport.

11. Abbreviations

Abbreviation	Meaning
AIS	Automotive Industry Standard
CPR	Cardiopulmonary Resuscitation

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.3.a	Commitment	Sections 3.1 and 5.1 address access to ambulance services commensurate with scope and ambulance level selection based on National Ambulance Code AIS-125.
COP.3.b	Commitment	Sections 3.2 and 5.2 address adequate ambulance access and space, quick exit and prominent signage.
COP.3.c	Commitment	Sections 3.3 and 5.3 address statutory compliance, at least basic life support equipment for adult and paediatric patients, and scope-based additional equipment. Stop-work authority (Section 6) also applies to this element.
COP.3.d	Commitment	Sections 3.4 and 5.4 address valid driving licence and basic life support/CPR training; additional crew may be present depending on situation and scope. Stop-work authority (Section 6) also applies to this element.
COP.3.e	Commitment	Sections 3.5 and 5.5 address daily vehicle/equipment/medication checks, post-trip medication top-up, verification and documentation, and sealed-kit checks where applicable. Stop-work authority (Section 6) also applies to this element.
COP.3.f	Commitment*	Sections 3.6 and 5.6 address the proper communication system covering the whole patient-transport process, written call/response guidance and predefined timeframe based on patient needs. Stop-work authority (Section 6) also applies to this element.

COP.3.g	Achievement	Sections 3.7 and 5.7 address early treatment opportunities while in transit, clinical information exchange and responsibility of the medical professional in the ambulance for transit care decisions.
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13. Required Records/Evidence Checklist

Ambulance access and statutory compliance

- Demarcated ambulance space with accessible patient-receiving area and quick exit.
- Prominent signage to ambulance entry and emergency department route.
- Applicable ambulance statutory documents, such as registration, fitness certificate, pollution control certificate and insurance.
- Documentation showing the ambulance level selected with reference to National Ambulance Code AIS-125.

Equipment and personnel

- Basic life support equipment for both adult and paediatric patients.
- Scope-based additional monitoring and resuscitative equipment.
- Valid driving licence for the ambulance driver.
- Basic life support and CPR training records for ambulance personnel.

Daily and post-trip checks

- Documented daily vehicle-system checks covering lights, siren, beacon lights, fuel and tyres.
- Documented medical-equipment checks.
- Daily emergency-medication availability and expiry checks.
- Post-trip medication top-up, verification and documentation.
- Sealed emergency medication-kit checks after each use where that system is used.
- Medication storage arrangements according to organisational policy.

Communication and transport

- Connection between ambulance and organisation/control room by wireless or mobile phones.
- Written call-receipt, response and transport-organisation guidance.
- Communication coverage for the whole patient-transport process.
- Predefined timeframe for ambulance departure based on patient needs.

In-transit care

- File created from first communication with the patient or attendant.

- Available clinical information such as age, weight, provisional diagnosis and ongoing treatment at the referring organisation.
- Information exchange between ambulance personnel and receiving medical professional when required.
- Attempts, wherever possible, by the receiving doctor to communicate with an external ambulance agency when applicable.
- Documentation identifying the medical professional in the ambulance responsible for care/intervention decisions during transit.

14. References

1. National Ambulance Code AIS-125.
2. Motor Vehicle Act and applicable statutory requirements for ambulance vehicles.
3. Guidebook interpretation supplied for COP.3.a through COP.3.g.

Disclaimer

This policy reorganises the supplied COP.3 objective elements and Guidebook interpretation into plain language. Mandatory requirements and their stated modal strength have been retained. Optional, preferable, illustrative and scope-dependent provisions have not been converted into mandatory requirements.